

GA Healthcare Training & Consulting

Nurse Aide Training Program

Student Handout Packet

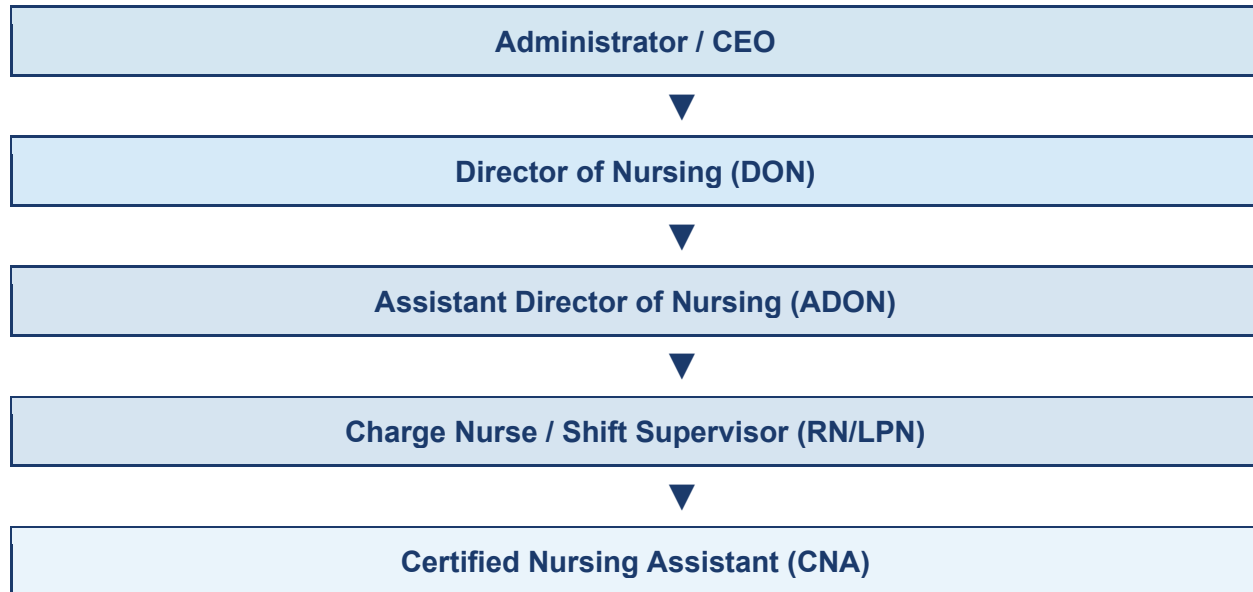
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H/O – Chain of Command Diagram

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Name: _____ Date: _____

Nursing Home Chain of Command



Escalation Process

Step	Action	Timeframe
1	Attempt to resolve directly with coworker	Immediately
2	Report to Charge Nurse / Shift Supervisor	Same shift
3	Notify Director of Nursing (DON)	If unresolved
4	Contact Administrator / Facility Management	Escalated issues

Key Principles

- Always report concerns to your direct supervisor first.
- Document all communications and follow up in writing when possible.
- Never skip levels of the chain of command without cause.
- In emergencies (resident safety), notify the charge nurse immediately.
- Never retaliate against a coworker for escalating a concern.

Reflection

Who is your direct supervisor in this facility?

Describe a situation where you would escalate beyond the charge nurse:

H/O – Standard Precautions Quick Reference

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Handwashing – When to Wash

- Before and after direct resident contact
- After removing gloves
- Before and after eating or preparing food
- After using the restroom
- After contact with blood, body fluids, or contaminated surfaces
- When moving from a contaminated to a clean area

Handwashing Steps (CDC Method)

1.	Wet hands with clean, running water (warm or cold).
2.	Apply soap and lather well, including backs of hands, between fingers, and under nails.
3.	Scrub for at least 20 seconds (hum 'Happy Birthday' twice).
4.	Rinse hands under clean, running water.
5.	Dry with a clean towel or air dry.
6.	Use the towel to turn off the faucet.

Personal Protective Equipment (PPE)

PPE Item	When to Use	Key Reminders
Gloves	Any contact with blood, body fluids, mucous membranes, or non-intact skin	Change between tasks; wash hands after removal
Gown	Anticipated contact with blood/body fluids or splashing	Remove before leaving room; gown is single-use
Mask	Procedures that may generate splashing or sprays	Ensure nose wire is fitted; dispose after use
Eye Protection	Risk of blood/fluid splash to eyes or face	Reusable goggles must be disinfected between uses

Donning (Putting On) PPE — In Order

- 1. Gown
- 2. Mask or Respirator
- 3. Eye Protection (goggles or face shield)
- 4. Gloves (pull over gown cuffs)

Doffing (Removing) PPE — In Order

- 1. Gloves (most contaminated — remove first)
- 2. Eye Protection
- 3. Gown
- 4. Mask (least contaminated — remove last)
- Wash hands immediately after each step.

Transmission-Based Precautions

Type	Route	Examples	Precautions
Contact	Direct/indirect touch	MRSA, C. diff, Wound infections	Gown + gloves; private room preferred
Droplet	Large droplets (>5 microns)	Influenza, COVID-19, Pertussis	Surgical mask; 3-ft distance minimum
Airborne	Small particles (<5 microns)	TB, Measles, Chickenpox	N95 respirator; negative pressure room

Quick Review Questions

1. List the 6 steps of handwashing in order:

2. Which PPE is removed first and why?

H/O – Hazard Identification Worksheet

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Name: _____ Date: _____

Instructions

During your facility walk-through, identify safety hazards in the environment. Record each hazard, its location, and the corrective action needed in the table below.

Environmental Safety Walk-Through Log

Hazard Observed	Location	Corrective Action Needed

Common Hazard Categories — Reference Checklist

Hazard Type	Examples to Look For
Fall Hazards	Wet floors, loose rugs, poor lighting, items in walkways
Equipment Hazards	Frayed cords, broken call lights, malfunctioning bed rails
Chemical/Sharps Hazards	Unsecured cleaning products, exposed sharps, unlabeled containers
Fire Hazards	Blocked exits, expired extinguishers, overloaded outlets
Infection Control Hazards	Improper waste disposal, soiled linen on floor
Ergonomic Hazards	Missing gait belts, improperly adjusted beds/chairs

Reflection

Which hazard surprised you most? How would you address it?

H/O – OBRA Residents' Rights Summary

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What Is OBRA 1987?

The Omnibus Budget Reconciliation Act of 1987 (OBRA) established federal standards for nursing home care, including a comprehensive set of rights for residents. CNAs must know, protect, and uphold these rights every day.

OBRA Residents' Rights Quick Reference

Right Category	What It Means for the CNA
Dignity & Respect	Address residents by preferred name; knock before entering; provide privacy during care.
Privacy & Confidentiality	Do not share resident health information; close curtains; use private spaces for conversations.
Personal Choices	Honor resident preferences in daily routines, clothing, meals, and activities when safe.
Freedom from Restraints	Never apply physical or chemical restraints without a physician's order and documented medical necessity.
Freedom from Abuse	Recognize and report all forms of abuse, neglect, exploitation, and mistreatment immediately.
Communication	Ensure residents can send/receive mail, make phone calls, and visit with family without interference.
Participation in Care	Involve the resident in their care plan; explain procedures before performing them.
Right to Complain	Support residents who raise grievances; never retaliate or discourage complaints.
Financial Security	Never handle resident money without authorization; report missing property immediately.
Access to Ombudsman	Inform residents of their right to contact the LTC Ombudsman without fear of reprisal.

The Role of the Long-Term Care Ombudsman

- An independent advocate for nursing home and assisted living residents.
- Investigates complaints made by or on behalf of residents.
- Can be contacted confidentially — without facility involvement if the resident chooses.
- CNAs must never discourage or obstruct a resident's access to the ombudsman.
- Georgia LTC Ombudsman Program: 1-888-454-5826

Reporting Rights Violations

- Report any suspected violation to the charge nurse immediately.
- Document what you observed — facts only, no opinions.
- Mandatory reporters are required by law — failure to report is a violation.
- Retaliation against a reporting CNA is prohibited.

Knowledge Check

1. List 4 rights protected under OBRA 1987:

2. What is the CNA's responsibility if a resident's rights are violated?

3. Can a CNA apply a restraint to prevent a fall without a physician's order?

H/O – Georgia CNA Scope of Practice

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What Is Scope of Practice?

Scope of practice defines the tasks and responsibilities a CNA is legally authorized to perform under Georgia law and the supervision of a licensed nurse. Performing tasks outside of scope can result in disciplinary action, loss of certification, or harm to residents.

Georgia CNA Scope of Practice

Within CNA Scope of Practice	OUTSIDE CNA Scope of Practice
✓ Measuring and recording vital signs	X Administering medications of any kind
✓ Assisting with ADLs (bathing, grooming, dressing)	X Performing sterile wound care or dressing changes
✓ Transporting residents; assisting with mobility	X Inserting or removing catheters
✓ Collecting specimens as directed by nurse	X Interpreting lab results or diagnostic findings
✓ Reporting observations and changes to the nurse	X Giving medical advice or diagnosis
✓ Performing ROM exercises as directed	X Initiating or discontinuing treatments
✓ Documenting care provided accurately	X Applying or removing restraints without order
✓ Supporting residents with nutrition and hydration	X Adjusting oxygen flow rates

Key Legal Principles

- Negligence: Failing to perform a duty a reasonable CNA would perform in a similar situation.
- Malpractice: A professional's deviation from the standard of care causing harm.
- False Imprisonment: Physically restraining or confining a resident without legal authority.
- Mandatory Reporting: CNAs are legally required to report suspected abuse, neglect, and exploitation.

HIPAA & Confidentiality

- Never share protected health information (PHI) without resident consent.
- Do not discuss residents in hallways, elevators, or public areas.
- Social media posts about residents — even without names — can violate HIPAA.
- PHI includes name, diagnosis, room number, and any identifying information.

Scenario Review

A nurse asks you to change a wound dressing while she helps another resident. What do you do?

List 3 tasks outside the Georgia CNA scope of practice:

H/O – Vital Signs Normal Ranges Reference Card

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Vital Signs at a Glance

Vital Sign	Normal Adult Range	Report If...	Notes
Temperature (Oral)	97.6°F – 99.6°F (36.5°C – 37.6°C)	Above 100.4°F (fever) or below 96°F	Axillary is 1° lower; Rectal is 1° higher
Pulse Rate	60 – 100 bpm	Below 60 or above 100 bpm; irregular rhythm	Count for a full 60 seconds if irregular
Respirations	12 – 20 breaths/min	Below 12 or above 20/min; labored, noisy, or absent	Count without telling the resident
Blood Pressure	Less than 120/80 mmHg	Above 140/90 (HTN); Below 90/60 (hypotension)	Take in same arm each time; resident rested
Oxygen Saturation (SpO ₂)	95% – 100%	Below 92% — notify nurse immediately	Report any drop from resident's baseline
Weight	Varies by resident	Gain or loss of 2+ lbs in 24 hrs or 5+ lbs in 1 week	Same scale, same time, same clothing each time

Temperature Methods

Method	Site	Add/Subtract	Use When
Oral	Under tongue	Standard (baseline)	Cooperative adults; no oral issues
Axillary	Under armpit	Add 1°F to convert to oral equivalent	Children; when oral route is contraindicated
Rectal	Rectum	Subtract 1°F to convert to oral equivalent	Accurate but used less; requires caution
Tympanic	Ear canal	Consult thermometer instructions	Quick; avoid if ear infection present

Blood Pressure Classification (AHA)

Category	Systolic (mmHg)	Diastolic (mmHg)
Normal	Less than 120	Less than 80
Elevated	120 – 129	Less than 80
High (Stage 1)	130 – 139	80 – 89
High (Stage 2)	140 or higher	90 or higher
Hypertensive Crisis	Higher than 180	Higher than 120
Hypotension (Low)	Less than 90	Less than 60

Documentation Practice

Record the following vital signs using correct format:

Temp (°F)	Pulse (bpm)	Respirations (bpm)	BP (mmHg)	SpO2 (%)

H/O – Bedmaking Checklist: Occupied & Unoccupied Bed

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Name: _____ Date: _____

Why Bedmaking Is a Tested Skill

A wrinkle-free, dry, properly made bed is a pressure-injury prevention measure, not housekeeping. Making an occupied bed also appears on the Georgia Required Skills Checklist and must be checked off at 100% in the lab before you may attend clinical. Reference: Hartman's Nursing Assistant Care: The Basics, 6th Edition — occupied bed, pp. 209–210; unoccupied bed, p. 213.

Unoccupied Bed — Procedure Steps

Procedure Step	Critical Point / Rationale
1. Wash your hands and gather clean linen.	Stack linen in the order of use so the bottom sheet is on top of the pile.
2. Carry linen away from your uniform.	Linen touching your uniform is considered contaminated.
3. Put on gloves if the bed linen is soiled.	Standard Precautions apply to all body fluids.
4. Raise the bed to a comfortable working height and lock the wheels.	Protects your back; prevents the bed from rolling.
5. Place the bed flat and lower the side rails.	Gives you a level surface to work on.
6. Loosen soiled linen and roll it away from you, dirty side inward.	Never shake linen — shaking spreads microorganisms through the air.
7. Place soiled linen in the hamper or laundry bag.	Never place linen on the floor, on a chair, or on another resident's bed.
8. Remove and discard gloves; wash your hands.	Do not touch clean linen with soiled gloves.
9. Place the mattress pad and bottom sheet, center fold at the middle of the bed.	Centering keeps the sheet even on both sides.
10. Miter the corners at the head and the foot; tuck in tightly.	A wrinkle-free surface helps prevent pressure injuries.
11. Add the draw sheet if used; tuck in on both sides.	The draw sheet is used to move and turn the resident.
12. Place the top sheet, blanket, and bedspread; center each one.	Hem side of the top sheet faces up, seams away from the resident's skin.
13. Make a toe pleat and miter the corners at the foot.	The pleat gives the feet room and helps prevent foot drop and skin breakdown.
14. Put a clean case on the pillow.	Never hold the pillow under your chin — that contaminates it.
15. Return the bed to its lowest position.	A low bed is the single most important fall precaution.
16. Wash your hands.	Ends every procedure.

Occupied Bed — Procedure Steps

Procedure Step	Critical Point / Rationale
1. Wash your hands and gather clean linen.	Bring everything you need before you start; never leave the resident to fetch linen.
2. Identify the resident and greet him or her by name.	Confirms you have the right resident and shows respect.
3. Explain what you are going to do before you begin.	Keep the resident informed throughout the entire procedure.
4. Provide privacy — curtain, door, and window shades.	Privacy is a Residents' Right, not a courtesy.
5. Put on gloves.	Bottom linen may be soiled with body fluids.
6. Raise the bed to a safe working height and lock the wheels.	Never leave the resident alone with the bed raised.
7. Lower the head of the bed flat if the resident can tolerate it.	Stop and reposition if the resident has trouble breathing.
8. Cover the resident with a bath blanket and remove the top sheet from underneath.	Keeps the resident warm and covered; prevents exposure.
9. Raise the side rail on the far side of the bed.	The raised rail keeps the resident from rolling off while turned away from you.
10. Loosen the bottom linen on your side and turn the resident toward the far rail.	Adjust the pillow under the head after turning.
11. Roll the soiled bottom linen toward the resident's back, soiled side inward.	Rolling inward contains the contamination.
12. Place clean bottom linen on the empty half; miter, tuck, and fanfold the rest against the resident's back.	Smooth every wrinkle — the resident is lying on this surface.
13. Raise your side rail, move to the other side, and lower that rail.	Both rails are never down at the same time with a resident in the bed.
14. Help the resident roll over the linen toward you onto the clean side.	Tell the resident there will be a bump under the hip.
15. Remove the soiled linen and place it in the hamper or bag.	Do not shake; do not let it touch your uniform.
16. Pull the clean linen through, smooth it, miter, and tuck.	Check for wrinkles one more time before repositioning.
17. Reposition the resident on the back in good body alignment.	Use pillows and supports as directed by the care plan.
18. Place the clean top sheet over the bath blanket, then pull the bath blanket out from underneath.	The resident is never uncovered during the change.
19. Make a toe pleat; miter the corners at the foot.	Loose top linen prevents pressure on the toes.
20. Change the pillowcase, supporting the resident's head.	Support the head and neck the whole time.
21. Remove gloves and wash your hands.	Remove gloves before touching clean surfaces.
22. Lower the bed, place the call light within reach, and open the curtain.	Side rails up only if the care plan directs it.
23. Report and record what you observed.	Report skin changes, drainage, incontinence, or pain right away.

Bed Types — Know the Difference

Bed Type	When It Is Used
Closed bed	Bedspread pulled all the way up over the pillow. Used when the room is empty or a new resident has not yet arrived.
Open bed	Top linens fanfolded down to the foot. Used when the resident is about to get back into bed.
Occupied bed	Made while the resident stays in the bed. Used when the resident cannot get up.
Surgical (recovery) bed	Top linens fanfolded to one side, bed left flat and raised. Used to receive a resident arriving on a stretcher.

Lab Skills Check-Off

Required Skill	Lab Practicum Sign-Off
Making an unoccupied bed — Hartman 6e, p. 213	Instructor initials / date: _____
Making an occupied bed — Hartman 6e, pp. 209–210	Instructor initials / date: _____

Reflection

Name two things you would do differently if the resident began coughing while turned onto one side.

H/O – Braden Scale Pressure Ulcer Risk Assessment

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What Is the Braden Scale?

The Braden Scale is a standardized tool used to identify residents at risk for developing pressure ulcers (pressure injuries). It evaluates 6 categories. Scores range from 6 to 23. Lower scores = higher risk.

Risk Category	Total Score	Risk Level	Action
Very High Risk	9 or less	Severe	Reposition every 1 hour; specialty mattress
High Risk	10 – 12	High	Reposition every 2 hours; pressure-relieving surface
Moderate Risk	13 – 14	Moderate	Reposition every 2–4 hours; monitor skin closely
Mild Risk	15 – 18	Mild	Reposition as tolerated; skin assessment each shift
No Risk	19 – 23	Minimal	Routine care; preventive positioning

Pressure Ulcer Stages

Stage	Description
Stage I	Non-blanchable redness on intact skin; area may be painful, firm, or warm.
Stage II	Partial thickness skin loss; appears as a shallow open ulcer or blister.
Stage III	Full thickness skin loss; subcutaneous fat may be visible; no bone, tendon, or muscle exposed.
Stage IV	Full thickness skin/tissue loss; bone, tendon, or muscle is exposed.
Unstageable	Full thickness loss; base covered by slough or eschar — cannot determine depth.
Deep Tissue Injury	Purple or maroon area of intact skin due to pressure/shear; may evolve rapidly.

CNA Prevention Responsibilities

- Reposition residents on a schedule per the care plan.
- Inspect skin during every bathing, transfer, and dressing activity.
- Report any redness, breakdown, or skin changes to the nurse immediately.
- Keep skin clean and dry; apply moisture barrier as directed.
- Ensure adequate nutrition and hydration to support skin integrity.
- Use protective devices (heel protectors, foam wedges) as ordered.

H/O – Bladder & Bowel Elimination Log

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Name: _____ Date: _____

Resident Name: _____ Room #: _____ Date: _____

Elimination Tracking Log

Time	Voided (U)	Incontinent (I)	BM	Amount/Notes	Fluid Intake (mL)	Staff Init.
6:00 AM						
7:00 AM						
8:00 AM						
9:00 AM						
10:00 AM						
11:00 AM						
12:00 PM						
1:00 PM						
2:00 PM						
3:00 PM						
4:00 PM						
5:00 PM						
6:00 PM						
7:00 PM						
8:00 PM						
10:00 PM						
TOTAL FLUID INTAKE:						

Legend & Documentation Guidelines

Code	Meaning
U	Resident voided (urinated) in toilet, commode, or bedpan — voluntary control
I	Incontinent episode — urine or stool released without resident control
BM	Bowel movement — note consistency using Bristol Stool Chart if facility requires
Init.	CNA's initials confirming documentation accuracy

What to Report to the Nurse

- No void for 8 or more hours
- Urine that is dark, cloudy, or foul-smelling
- Burning or pain during urination (report immediately if resident communicates this)
- No BM for 3 or more days
- Sudden increase in incontinence episodes
- Blood in urine or stool

H/O – Clinical Observation & Reporting Checklist

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Name: _____ Date: _____

Resident Room #: _____ Clinical Site: _____ Shift: _____

Cardiorespiratory

✓	Observation/Action	Time/Notes
☐	Shortness of breath or labored breathing	
☐	Rapid or abnormal respirations	
☐	Chest pain or pressure reported by resident	
☐	Lips, fingertips, or skin appear bluish (cyanosis)	
☐	New or worsening cough	
☐	Fever or chills present	

Gastrointestinal & Urinary

✓	Observation/Action	Time/Notes
☐	Abdominal pain or nausea reported	
☐	Vomiting observed	
☐	Excessive thirst or sweating	
☐	Blood or sediment visible in urine	
☐	Difficulty or pain when urinating	
☐	Frequent urination in small amounts	
☐	Urine appears dark or has strong odor	
☐	No bowel movement in 3+ days	

Behavioral & Functional Changes

✓	Observation/Action	Time/Notes
☐	Resident appears more agitated or anxious than baseline	
☐	Reduced communication or verbal response	
☐	Visible changes in physical appearance or hygiene	

✓	Observation/Action	Time/Notes
☐	Refused to participate in activities	
☐	Appears unusually tired, weak, confused, or drowsy	
☐	Requires more assistance with ADLs than usual	

Nutrition, Hydration & Weight

✓	Observation/Action	Time/Notes
☐	Eating less than 50% of meals	
☐	Drinking less than usual	
☐	Weight change of 2+ lbs in 24 hours or 5+ lbs in 1 week	
☐	Difficulty chewing or swallowing observed	

Skin

✓	Observation/Action	Time/Notes
☐	New area of redness that does not blanch	
☐	Open wound, blister, or skin tear observed	
☐	Skin color change — jaundice, pallor, or mottling	
☐	Swelling or edema in extremities noted	

Reporting Summary

Concerns reported to: _____ Time reported: _____ Method: _____

Summary of what was reported:

H/O – Georgia CNA Registry Access Guide & Renewal Checklist

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What Is the Georgia Nurse Aide Registry (NAR)?

The Georgia Nurse Aide Registry is a state-maintained database of all individuals who have successfully completed a state-approved nurse aide training program and passed the required competency evaluation. All CNAs in Georgia must be listed on the NAR to work legally in a Medicare/Medicaid-certified facility.

How to Access the Georgia NAR

1.	Visit the Georgia NAR website: https://nurseaide.allianthealth.org
2.	Select 'Search for a Nurse Aide' from the registry menu.
3.	Search by your full legal name or certificate number.
4.	Confirm your listing shows 'Active' status.
5.	Print or save your certification status for employment records.
6.	For phone verification: Call 678-527-3010, Option 4.

Georgia CNA State Exam Overview

Component	Details
Testing Vendor	Credentia (credentia.com/test-takers/ga)
Written/Oral Exam	70 multiple-choice questions; passing score is 70%
Clinical Skills Exam	5 skills selected randomly; all critical elements must be performed correctly
ID Required	Two forms of valid ID (one must be government-issued with photo)
Scheduling	Register online via your CNA365 account at credentia.com/test-takers/ga
Attempts	3 attempts for each part (Written and Skills). Both parts must be passed within one (1) year of program completion — Eligibility Route E1, New Nurse Aide. Failure to do so requires retraining at a Georgia-approved NATP.

Certification Renewal Checklist

	Renewal Requirement	Status / Date
☐	Verify active employment — CNAs must work at least 8 hours of nursing-related paid work per 2-year renewal period.	
☐	Confirm employer has filed required documentation with the NAR.	
☐	Check that 12 hours of in-service training have been completed annually.	
☐	Verify no findings of abuse, neglect, or misappropriation on your NAR record.	
☐	Update your address with the NAR if you have moved.	
☐	Obtain a copy of your certification for your personal records.	

H/O – Employment Resources & Advancement Pathways

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Resume Tips for New CNAs

- Lead with your CNA certification and Georgia NAR status.
- Highlight clinical skills: vital signs, ADL assistance, transfers, dementia care.
- Include any certifications (CPR/BLS, First Aid, FBAO).
- List clinical training site and dates completed.
- Use action verbs: assisted, monitored, documented, communicated, demonstrated.
- Keep resume to 1 page; proofread carefully for spelling and grammar.

Where to Find CNA Jobs in Georgia

Resource	Notes
Georgia Department of Labor	gdol.ga.gov — job listings, resume help, career centers
Indeed / LinkedIn / ZipRecruiter	Search 'CNA Georgia' + your city; set alerts
Nursing Home / LTC Facilities	Apply directly to facilities in your area
Staffing Agencies	CNA staff agencies offer flexible scheduling and varied settings
Hospital Systems	Many hospitals hire CNAs for patient care tech/hospital aide roles
Home Health Agencies	Provide in-home care; may offer part-time/flexible hours

Career Advancement Pathways

Role	Education Required	Next Steps
CNA I (Current)	Nurse Aide Training Program + State Exam	Active NAR listing; CPR certification
CNA II	Advanced CNA training + additional competency testing	Check with employer for sponsorship opportunities
Medication Aide	State-approved medication aide course	Available after 1+ year as CNA in Georgia
LPN (Licensed Practical Nurse)	Approved PN program (12–18 months) + NCLEX-PN	Many community colleges offer bridge programs
RN (Registered Nurse)	ADN (2 yrs) or BSN (4 yrs) + NCLEX-RN	LPN-to-RN or CNA-to-RN bridge pathways available
BSN / Advanced Practice	BSN then MSN or DNP programs	Specialties: NP, CRNA, CNM, CNS, education, leadership

Professional Organizations to Join

- National Black Nurses Association (NBNA) – nbna.org
- Georgia Nurses Association (GNA) – georgianurses.org
- American Nurses Association (ANA) – nursingworld.org
- National Association of Health Care Assistants (NAHCA) – nahcacares.org

My Career Goals — Personal Action Plan

My first CNA job goal (setting, city, timeline):

My long-term healthcare career goal:

One action I will take in the next 30 days toward my career: